



FENA SUSPENSION 30MG/5ML

Description: Pearl white colour, blackcurrant flavoured suspension.

Content:

Each 5ml contains: Fexofenadine hydrochloride 30mg (eq. to fexofenadine 28mg)

Preservatives: methyl parabens 5.0mg/5ml and propyl parabens 0.5mg/5ml

Pharmacology:

Wheal and Flare: Human histamine skin wheal and flare studies in adults following single and twice daily doses of 20 and 40 mg fexofenadine hydrochloride demonstrated that the drug exhibits an antihistamine effect by 1 hour, achieves maximum effect at 2 to 3 hours, and an effect is still seen at 12 hours. There was no evidence of tolerance to these effects after 28 days of dosing. The clinical significance of these observations is unknown. Histamine skin wheal and flare studies in 7 to 12 years old subjects showed that following a single dose of 30 or 60 mg, antihistamine effect was observed at 1 hour and reached a maximum by 3 hours. Greater than 49% inhibition of wheal area, and 74% inhibition of flare area were maintained for 8 hours following the 30 and 60mg dose. No statistically significant increase in mean QTc interval compared to placebo was observed in 714 adult subjects with seasonal allergic rhinitis given fexofenadine hydrochloride capsules in doses of 60 to 240mg twice daily for 2 weeks. Pediatric subjects from 2 placebo controlled trials (n=855) treated with up to 60 mg fexofenadine hydrochloride twice daily demonstrated no significant treatment- or dose-related increases in QTc. In addition, no statistically significant increase in mean QTc interval compared to placebo was observed in 40 healthy adult subjects given fexofenadine hydrochloride as an oral solution at doses up to 400 mg twice daily for 6 days, or in 230 healthy adult subjects given fexofenadine hydrochloride 240 mg once daily for 1 year. In subjects with chronic idiopathic urticaria, there were no clinically relevant differences for any ECG intervals, including QTc, between those treated with fexofenadine hydrochloride 180 mg once daily (n = 163) and those treated with placebo (n = 91) for 4 weeks.

The pharmacokinetics of fexofenadine hydrochloride in subjects with seasonal allergic rhinitis and subjects with chronic urticaria were similar to those in healthy subjects. **Absorption:** A dose of 5ml of Fena suspension containing 30 mg of fexofenadine hydrochloride is bioequivalent to a 30 mg dose of tablet. Following oral administration of a 30mg dose of Fena suspension to healthy adult subjects, the mean C_{max} was 118 ng/mL and occurred at approximately 1 hour. The administration of 30mg Fena suspension with a high fat meal decreased the AUC and the mean C_{max} by approximately 30 and 47%, respectively in healthy adult subjects. **Distribution:** Fexofenadine hydrochloride is 60% to 70% bound to plasma proteins, primarily albumin and 1-acid glycoprotein. **Metabolism:** Approximately 5% of the total dose of fexofenadine hydrochloride was eliminated by hepatic metabolism. **Elimination:** The mean elimination half-life of fexofenadine was 14.4 hours following administration of 60 mg twice daily in healthy adult subjects. Human mass balance studies documented a recovery of approximately 80% and 11% of the [14C] fexofenadine hydrochloride dose in the feces and urine, respectively. Because the absolute bioavailability of fexofenadine hydrochloride has not been established, it is unknown if the fecal component represents primarily unabsorbed drug or is the result of biliary excretion. **Special Populations:** Pharmacokinetics in renally and hepatically impaired subjects and geriatric subjects, obtained after a single dose of 80 mg fexofenadine hydrochloride, were compared to those from healthy subjects in a separate study of similar design. **Renally Impaired:** In subjects with mild to moderate (creatinine clearance 41-80 ml/min) and severe (creatinine clearance 11-40 ml/min) renal impairment, peak plasma concentrations of fexofenadine were 87% and 111% greater, respectively, and mean elimination half-lives were 59% and 72% longer, respectively, than observed in healthy subjects. Peak plasma concentrations in subjects on dialysis (creatinine clearance ≤10 ml/min) were 82% greater and half-life was 31% longer than observed in healthy subjects. Based on increases in bioavailability and half-life, a dose of 60 mg once daily is recommended as the starting dose in adult patients with decreased renal function. For pediatric patients with decreased renal function, the recommended starting dose of fexofenadine is 30 mg once daily for patients 2 to 11 years of age and 15 mg once daily for patients 6 months to less than 2 years of age [see Administration]. **Hepatically Impaired:** The pharmacokinetics of fexofenadine hydrochloride in subjects with hepatic impairment did not differ substantially from that observed in healthy subjects. **Geriatric Subjects:** In older subjects (≥65 years old), peak plasma levels of fexofenadine were 99% greater than those observed in younger subjects (<65 years old). Mean fexofenadine elimination half-lives were similar to those observed in younger subjects. **Effect of Gender:** Across several trials, no clinically significant gender-related differences were observed in the pharmacokinetics of fexofenadine hydrochloride.

Indication:

Seasonal Allergic Rhinitis: Fena Suspension is indicated for the relief of symptoms associated with seasonal allergic rhinitis in children 2 to 11 years of age. **Chronic Idiopathic Urticaria:** Fena Suspension is indicated for treatment of uncomplicated skin manifestations of chronic idiopathic urticaria in children 6 months to 11 years of age.

Dosage and administration:

For oral use. Shake the bottle well before administering Fena Suspension 30mg/5ml.

Seasonal Allergic Rhinitis: Children 2 to 11 years: The recommended dose is 30mg(5ml) twice daily. A dose of 30mg(5ml) once daily is recommended as the starting dose in pediatric patients with decreased renal function.

Chronic Idiopathic Urticaria: Children 6 months to less than 2 years: The recommended dose is 15mg(2.5 ml) twice daily. For pediatric patients with decreased renal function, the recommended starting dose is 15mg(2.5ml) once daily. Children 2 to 11 years: The recommended dose is 30mg(5ml) twice daily. For pediatric patients with decreased renal function, the recommended starting dose is 30mg(5ml) once daily.

Contraindications:

It is contraindicated for use in patients with hypersensitivity to any of its formula components.

Warnings:

The efficacy and safety of fexofenadine hydrochloride have not been established in children under 2 years of age for seasonal allergic rhinitis and in children under 6 months of age for chronic idiopathic urticaria. Dosage adjustment for the elderly or patients with hepatic failure is not required.

Risk of using the medication via not recommended routes of administration

No studies have been conducted on the effects of Fena Suspension when taken via not recommended routes of administration. Therefore, for your own safety and for the best efficacy of this medication, the drug must only be taken orally.

Drug Interactions:

The concomitant administration of fexofenadine hydrochloride with erythromycin or ketoconazole did not demonstrate any significant increase in the QTc interval. No difference in adverse effects was reported when these agents were administered combined or separately. No fexofenadine-omeprazole interaction was observed. However, the administration of an antacid containing aluminum hydroxide and magnesium, approximately 15 minutes before receiving fexofenadine hydrochloride reduced bioavailability. It is recommended to allow an interval of approximately two hours between administrations of fexofenadine hydrochloride and antacids that contain aluminium hydroxide and magnesium.

Use in pregnancy and lactation:

No studies of Fena Suspension have been conducted in pregnant women and breast feeding women. It should not be used during pregnancy unless the potential benefits exceed the potential risks to the fetus. In studies that assessed reproductive toxicity conducted in mice, fexofenadine did not affect fertility, was not teratogenic and did not affect pre or postnatal development. It must only be used by lactating women if the potential benefits exceed the potential risks to the child. This medication must not be used by pregnant women without a physician or surgeon dentist's advice. Risk category in pregnancy: C.

Adverse reactions:

In placebo-controlled studies involving patients with seasonal allergic rhinitis and chronic idiopathic urticaria, adverse events were similar to those of patients treated with placebo or fexofenadine. Most frequently reported adverse events by adults include: headache (>3%), drowsiness, dizziness and nausea (1-3%). The adverse events reported during the controlled studies involving patients with seasonal allergic rhinitis and chronic idiopathic urticaria, with an incidence lower than 1% and similar to those of the placebo group and that were rarely reported after commercialization include: fatigue, insomnia, nervousness, alteration in sleep quality or nightmares. Patients also reported rare cases of exanthema, urticaria, itching and hypersensitive reactions, such as: angioedema, thoracic rigidity, dyspnea, redness and systemic anaphylaxis. The adverse events reported in placebo-controlled studies of chronic idiopathic urticaria were similar to those reported in placebo-controlled studies of allergic rhinitis. In placebo-controlled studies in children from 6 to 11 years of age with seasonal allergic rhinitis, adverse events were similar to those observed in clinical studies involving adults and children 12 years of age or older with seasonal allergic rhinitis. In clinical controlled studies involving pediatric patients from 6 months to 5 years of age, no unexpected adverse events were observed in patients treated with fexofenadine hydrochloride.

Treatment of over dosage:

Symptoms: Dizziness, drowsiness and dry mouth were reported.

Treatment: In case of overdose, it is recommended that the usual symptomatic and support measures be taken to remove the non-absorbed drug from the body. Hemodialysis does not effectively remove fexofenadine hydrochloride from the blood.

Effects on ability to drive and use machine

Studies conducted with fexofenadine hydrochloride did not associate the use of the product with reduced attention when operating motor vehicles or machinery, alteration in sleep quality or other effects on the central nervous system.

Storage conditions : Store in a cool dry place, below 30°C. Keep out of reach of children.

Shelf life : 12 months

Presentation : 60ml/bottle

Product registration holder &: PRIME PHARMACEUTICAL SDN. BHD.

Manufacturer 1505, Lorong Perusahaan Utama 1, Taman Perindustrian Bukit Tengah, 14000 Bukit Mertajam, Penang, Malaysia.

02/05/2025